

Standing Rock Service Unit (SRSU)

Performance Work Statement (PWS)

- 1.0 General:** This performance work statement describes the requirements for a **Non-personal service nursing services (Emergency Department)** to support the mission of the Indian Health Service (IHS).
- 1.1 Background:** IHS is an agency within the U.S. Department of Health and Human Services and is responsible for providing federal health services to American Indians and Alaska Natives. The provision of health services to members of federally recognized tribes grew out of the special government to government relationship between the federal government and Indian tribes. The IHS is the principal federal health care provider and health advocate for the Indian people. The goal is to assure that comprehensive, culturally acceptable personal and public health services are available and accessible to American Indians and Alaska Natives. The IHS currently provides health services to approximately 1.5 million American Indians and Alaska Natives who belong to more than 557 federally recognized tribes in 34 states. The Fort Yates Hospital (FYH) is a Level IV Trauma Center, 12-bed critical access hospital, located in rural, ND.
- 1.2 Scope:** The Contractor shall provide nursing services in accordance with section 5.0, performance based requirements.
- 1.2.1 Duties and responsibilities** will generally focus on emergency department services, but may encompass, inpatient nursing services at the facility.
- 1.2.2 Place of Performance:** 10 Standing Rock Ave, Fort Yates, ND 58538
- 1.2.3 Period of Performance:** 1, 14 month base period with 4 1 year (12 month) option periods.
- 1.2.4 Base year 14 months: 4-1-26 to 5-31-27**
- A. Transition Period 4-1-26 to 5-31-26**
1. The transition period will allow contract to onboard, credential, and orient staff prior to a full transition from previous vendor. Prior to 5-31-26 contract must provide requested staff based off current need who have rotated onsite for orientation to include hands-on training with EHR system. It is required that a soft-hand off/trial vendor switch will happen at minimum two (2) days prior to the official transition date with the current vendor onsite as backup only.
- B. Nursing Services 6-1-26 to 5-31-27 24960 Hours**
1. Ther service component is maximum of 24960 (8 nurses x 60 hours/week x 52 weeks/year) hours for continuous ED coverage. The government shall request through task orders the needed staff for appropriate levels. It is anticipated the government will request task orders for services 2-3 months on a task order on a rolling basis for the service period. The amount of nurses/hours requested per task order will vary due to permanent staffing vacancy rate.
- C. Minimum (2 month transition period)**

D. Exercising Option Periods- Due to the historical recruiting difficulties and unpredictable staffing patterns, the expected Notice of Intent to Exercise Option Periods is fifteen (15) days prior to the start of the next option period.

- 1.2.5. Option Year 1 12 months 6-1-27 to 5-31-28
- 1.2.6. Option Year 2 12 months 6-1-28 to 5-31-29
- 1.2.7. Option Year 3 12 months 6-1-29 to 5-31-30
- 1.2.8. Option Year 4 12 months 6-1-30 to 5-31-31
- 1.2.9. Transition Period: The government reserves the right not to exercise any or all option periods including due to providing services with a new vendor. It is anticipated that any contracting change will require a transition period and multiple vendors may be onsite training new contract staff. Contractor must still provide services until end of contract period during this transitional period.

1.3. **Applicable Documents:** Please see the web link listed unless document is listed as an attachment.

- 1.3.1. The Joint Commission on Accreditation of Healthcare Organizations (The Joint Commission) <http://www.jointcommission.org>
- 1.3.2. Centers for Medicare and Medicaid Services (CMS) Standards <http://www.cms.hhs.gov>
- 1.3.3. IHS Manual- "Health Care Service Standards"
- 1.3.4. Health Insurance Portability and Accountability Act (HIPAA) of 1996. <http://www.cms.hhs.gov/hipaa>
- 1.3.5. Privacy Act of 1974. <http://www.usdoj.gov/oip/privstat.htm>
- 1.3.6. Revised American Nurses Association Code of Ethics and Standards of Practice and Care (1996) <http://www.nursingworld.org>
- 1.3.7. State Nurse Practice Act for the Contractor's licensing state. http://medi_smart.com/practice.htm
- 1.3.8. IHS Service Unit and Health Center Policies, Procedures and Protocols. (See section 11.0 for a list of attachments and exhibits)

2.0 **Definitions**

- 2.1. **Acceptance:** Constitutes acknowledgement that the supplies or services conform to the applicable contract quality and quantity requirements, except as provided in FAR subpart 46.5 and subject to other terms and conditions of the contract.
- 2.2. **Approval:** Acknowledgment by the designated Government official that submittals, deliverables, or administrative documents (e.g., insurance certificates, installation schedules, planned utility interruptions, etc.) conform to the contractual requirements. Government approval does not relieve the Contractor from responsibility for compliance with contract requirements.
- 2.3. **Code of Ethics:** The Revised American Nurses Association Code of Ethics and Standards of Practice and Care, published in 1996 by the American Nurses Association which makes explicit primary goals, values and obligations of the nursing profession.

- 2.4. **Contracting Officer (CO):** A Government employee with the authority to enter into, administer, and/or terminate contracts and make related determinations and findings.
- 2.5. **Contractor:** The individual awarded a legal binding contract to provide supplies and services.
- 2.6. **Contracting Officer's Representative (COR) or Project Officer (PO):** A federal employee who assists the ordering/issuing activity contracting officer in the administration of task orders issued under this contract. The COR is primarily responsible for the technical assistance and day-to-day program management of the ordering activity's task orders. Ordering activities may have different designators for this employee (e.g. GTR - Government Technical Representative, COTR-Contracting Officer's Technical Representative, or PO_ Project Officer}.
- 2.7. **Cooperative Attitude:** Behavior that is positive and displays a willingness to perform assigned patient care tasks and to be a team player.
- 2.8. **Cultural Awareness:** Realization and respect for American Indian and Alaska Native practices.
- 2.9. **Customer:** Patients, staff and visitors of an IHS service unit and health center.
- 2.10. **Customer Evaluation/Input:** Written comments made to the Contracting Officer regarding the Contractor's performance. This is one of the criteria used to evaluate the Contractor's performance.
- 2.11. **Dependability:** Qualities of being trusted and being able to repeat the same task to yield the same result.
- 2.12. **Federal Acquisition Regulation (FAR):** The FAR is the primary regulation for use by all Federal Executive agencies in their acquisition of supplies and services with appropriated funds.
- 2.13. **Nonpersonal Services:** A contract under which the personnel rendering the services are not subject, either by the contract's terms or by the manner of its administration, to the supervision and control usually prevailing in relationships between the Government and its employees, as defined in FAR 37.
- 2.14. **Ordering Activity:** An authorized user of IHS that may issue a task order to obtain required services under this contract.
- 2.15. **Ordering Activity Contracting Officer:** A Government employee of IHS authorized and warranted to issue task orders and to make subsequent task order modification(s) under this contract. The Ordering Activity CO has the authority to make initial determinations on all matters of dispute regarding task orders.
- 2.16. **Orientation:** An activity designed to provide basic familiarization of the facility and transition the nurse into the IHS Service Unit and/or Health Center and the nursing unit where the services will be provided.
- 2.17. **Past Performance Information:** Relevant information regarding a contractor's actions under previously awarded contracts. This includes the contractor's record of conformance to

specifications and to standards of good workmanship; the contractor's record of containing and forecasting costs on any previously performed cost reimbursable contracts; the contractor's adherence to contract schedules, including the administrative aspects of performance; the contractor's history for reasonable and cooperative behavior and commitment to customer satisfaction; and generally, the contractor's business-like concern for the interest of the customer.

- 2.18. **Patient Outcome:** End result of nursing care.
- 2.19. **Performance based QASP Matrix:** Lists the services to be monitored and the standards to be applied.
- 2.20. **Personal Protective Equipment (PPE):** The equipment used to protect medical personnel from exposure to biological, chemical, and radioactive hazards.
- 2.21. **Service Unit:** The local administrative unit of IHS.
- 2.22. **Standards of Practice and Standards of Care:** Authoritative statements by which the nursing profession describes the responsibilities for which its practitioners are accountable. The standards provide direction for professional nursing practice and a framework for the evaluation of practice. The standards of professional nursing practice may pertain to general or specialty practice.
- 2.23. **Task Order:** An order issued in accordance with the terms of the contract that details an ordering activity's specific requirements. An ordering activities written order to obtain services, at a minimum will include the following where applicable: description of services, skill categories, hours, price, period of performance, contract number, and the ordering activities task order number.
- 2.24. **Technical Direction:** A directive to the contract nurses that provides medical tasks and assignments within a normal medical team environment. This may come from a medical provider or federal nursing supervisor.
- 2.25. **Tour of Duty:** The time of day the nurse is scheduled to perform nursing care duties; also considered the shift of the day, The time can vary according to the needs of each facility and/or clinic, e.g. 12 hour Tour of Duty, 8 hour Tour of Duty, 10 hour Tour of Duty.
- 2.26. **Valid Patient Complaint:** Justifiable accusation made by a patient and supported by investigation.
- 2.27. **Verifiable Emergency:** An unexpected/unplanned absence by the contractor requiring valid documentation to confirm the occurrence.

3.0 Government Furnished Information, Property and Services

- 3.1. **Information:** Government unique information related to this requirement, which is necessary for Contractor performance, will be made available to the Contractor. The Contracting Officer or designee will be the point of contact for identification of any required information to be

supplied by the Government.

- 3.2. **Joint Use by the Government and the Contractor:** Except for the property and service listed in 3.3 and 4.0, the Government will provide, for joint use by the Government and the Contractor, all necessary equipment, supplies, and clinic space to perform the services under this contract.

3.3. **Contractor Exclusive Use:**

3.3.1. **Personal Protective Equipment (PPE):** The Government will furnish the Contractor with appropriate PPE other than specified in paragraph 4 of the contract. The Government will be responsible for any repair, cleaning, and inventory required for the PPE. This does not include any type of uniform or laboratory coat.

3.3.2. The Government will provide facility specific contractor identification badges for each contractor. A minimum fee of \$100.00 will be charged for lost or destroyed badges.

- 3.4. **Training:** Facility specific orientation/training necessary for the Contractor to perform the required duties, e.g., IHS information technology (IT) systems and operational procedures. Training will be provided **ONLY** if the subject matter is necessary to improve or enhance the quality of nursing services or includes mandates made by the IHS Service Units/Health Centers while the nurse is working under this contract. Training **will not** be provided for the purpose of continuing education, career development or individual development.

- 3.5. **Supervisor:** The contractor nurses are subject to the supervision and direction of Director of Nursing or her designee including but not limited to, Assistant Director of Nursing, Emergency Department Nursing Supervisor, and Chief of Staff.

4.0 **Contractor Furnished Property**

- 4.1. Except for the property specified in paragraph 3 as government furnished, the Contractor shall provide all uniforms and other personal medical instruments subject to the following:

4.1.1. **Uniforms and Lab Coats:** Uniforms shall conform to the requirements uniform cloth policy

4.1.2. **Other personal medical instruments:** "Other personal medical instruments" are defined as Contractor owned items may include but not limited to stethoscope, scissors, as appropriate to the work unit. The Contractor shall not use unsafe equipment or supplies at any time during performance of this contract. All Contractor furnished equipment and supplies shall be subject to inspection by the Government and must be approved by the COR prior to use by the Contractor. The Government reserves the right to prohibit the use of any materials, supplies, or equipment.

- 5.0 **Performance Based Requirements:** The Contractor shall provide Emergency Department nursing services in the delivery of patient care to the Indian Health Service. Specific tasks include the following:

5.1. **Contract Administration:**

- 5.1.1. Contractor shall provide at time of bid and throughout entire contract a point of contact and alternative point of contact for:

- A. Coordination of credentialing and privileging application review and submission
 - B. Coordination of weekly timecard verification and approvals
 - C. Reviewing and approving invoices and supporting documents and other payment questions
 - D. Providing management and oversight of ED nurses
 - E. Submission of weekly/monthly staffing rosters
- 5.1.2. The contractor shall convene a contract kick-off meeting with the COR and necessary IHS officials, no later than 5 business days from the effective date of this contract. The contractor shall identify immediate actions necessary in order to implement the PWS, identify key personnel for accomplishing each task, a timeline for completion, and identifying any challenges that may impede success.
- 5.1.3. The contractor shall participate in weekly status update calls with IHS and other meetings as needed to discuss contract implementation issues and challenges. The contractor shall submit agenda items in advance of the calls. The Contractor shall maintain an ongoing Action Item list with updated status on each Action Item. The Action Item list shall maintain all completed and ongoing Action Items with related completion target dates.
- 5.1.4. The contractor shall provide 24/7 contact information including name, phone, and email for emergent staffing situations.
- 5.1.5. The contractor shall submit an ED Nursing Staff Management Plan based on the start dates defined in the contract award within five (5) business days of the contract award. The contractor shall provide full staffing coverage at all times. See 5.3.6 Contingency Plan.
- 5.1.6. Staffing and Recruiting
 - A. The contractor shall have new suitable staff available for submission within 1 week of requesting. Staff must be able to be onsite within 1 month of requesting pending no government delays in the background
 - B. Potential candidates must have any requested/needed documents returned within 1 week.
- 5.2. **Nursing Duties:**
 - 5.2.1. The Contractor shall perform nursing duties and manage patient's consistent with a rural critical access hospital emergency department identified by the Service Unit and as directed by the Supervisor.
 - 5.2.2. The Contractor shall perform in accordance with the competency standards listed in Exhibits, paragraph 11.
 - 5.2.3. The contractor shall provide professional nursing service in direct patient care as follows:
 - A. Contractor shall provide full range of emergency nursing care to patients at the FYH Emergency Department.
 - B. Contractor shall exercise knowledge of the nursing process and its application to clinical practice in order to provide individualized, goal-oriented nursing care to the patient(s).
 - C. Contractor shall exercise knowledge of scientific and psychosocial nursing theory, principles, practices and techniques in order to plan and provide quality nursing care which meets the physical, spiritual, and social-cultural needs of the Indian patient.
 - D. Contractor shall exercise knowledge of biological aspects of disease processes and illness in order to recognize Complications and implement appropriate therapeutic

intervention required.

- E. Contractor shall exercise knowledge of the medications and dosages in order to recognize their desired effects, side-effects, and complications of their use.
- F. Contractor shall analyze the results of various medical procedures and translate them into appropriate nursing interventions.
- G. Contractor shall **work** effectively and harmoniously within the health care team and with Indian patients and their families.
- H. Provide appropriate documentation regarding care rendered using Standing Rock Service Unit Electronic Health Record (EHR) system. All documentation by the provider shall be entered electronically using the Electronic Health Record system. The provider is responsible for utilizing the SRSU EHR system to obtain pertinent patient information in assisting care. It is the responsibility of the provider to use the training resources provided by the SRSU to develop competency in utilizing the Electronic Health Record system.
 - I. Follows facility policies and procedures, TJC, OSHA, Indian Health Services IHS and other federal licensing, accreditation and regulating body standards.
- J. Provides guidance to other staff regarding clinical decisions. Assist with and ensures unit is in compliance with daily quality checks.
- K. Contractor shall make patient unit medical, specifically deciding based on acuity. Contractor shall ensure any assigned daily checks and logs are completed in a timely manner.

5.1.4 Contractor shall be subject to peer review per Joint Commission standards.

5.1.5 Contractor shall meet the Safety management reporting requirements in accordance with the Service Unit Safety Measure Policy.

5.1.6 Contractor shall be responsible for all applicable Federal, State, and Local Taxes, Meals Travel/Transportation, and housing. The Contractor shall be responsible and liable for the proper care of government owned or leased property.

5.2 **Hours of Operation**

5.2.1. Emergency Department is open 24 hours a day, 7 days a week, 365 days a year.

5.3 **Work Schedule:**

5.3.1. **Work shift:** Shifts may vary including 8, 10, 12 hour shifts pending availability of permanent staff.

5.3.2. The Emergency Department Supervisor shall determine specific tour of duty. The contractor shall work 100% of the contracted hours.

5.3.3. **Work Flexibility.** As directed by the Emergency Department Supervisor or designee, the Contractor shall assist the inpatient department as needed to support patient care.

- 5.3.4. The Contractor shall obtain approval of the department Supervisor or designee prior to any absence from work. If the length of the absence exceeds eight (8) work hours the Contractor shall request approval at least fifteen (15) days, (the exception is verifiable emergencies) in advance of the desired absence.
 - 5.3.5. Approval of leave is contingent upon availability of a qualified replacement.
 - 5.3.6. **Contingency Plan:** Continuity of care is an essential part of FYH medical services. The contract shall have a contingency plan in place to be utilized if nurse(s) leaves the Contractor's employment or is unable to continue performance in accordance with the terms and conditions of the resulting contracting. The contracting company shall maintain credentialed and background cleared employees on standby addition to key personnel working the facility.
 - 5.3.7. **Key Personnel:** Contract staff currently working at the facility. It is essential that continuity of services is maintained to the maximum degree possible, hence, substitution of contractor provided nurses shall be limited to urgent/emergent absences of approved, assigned nurses. The contractor shall be responsible for providing coverage to the Fort Yates hospital during periods of vacancies of the contractor's personnel due to sick leave, personal leave, vacations and add additional coverage as required.
- 5.3. **Conduct:** The Contractor shall meet standards as listed in the Federal Code of Conduct.
- 5.3.1. The contractor shall be subject to character investigation as required by Public Law 101-360, the Indian Health Child Protection and Family Violence Prevention Act prior to performance of a contract by utilizing the US Office of Personnel Management (OPM), Electronic Questionnaire for Investigation Processing (eQIP) system and this shall include all fingerprinting procedures and clearances. The contractor shall provide all requested information necessary to perform Level I and Level II background checks. The contractor shall comply with the requirement to obtain security investigations. The contractor shall work with the I.H.S. to ensure that the pre-employment screening process includes the appropriate investigation questionnaires and forms to be completed. All completed forms will be reviewed by the contractor and forwarded to the government personnel. The contractor will be immediately removed from the position if at any time the investigation receives unfavorable adjudication, or, if other unfavorable information that would affect the investigation becomes known.
- 5.4. **Performance Evaluation:**
- 5.4.1. The Contractor's performance will be evaluated in accordance with the standards set forth in the contract and Performance-based QASP Matrix of section 10.0.
 - 5.4.2. Substantiated reports written by any customer dealing with patient safety, infection control, or other procedure that adversely affects patient outcome constitutes a breach of contract.
- 5.5. **Identification of Contractor:**
- 5.5.1. The Contractor shall wear a government issued contractor identification badge during performance of duty. PIV cards are issued pending HR policy.
- 5.6. **Management of Medical Information:** The Contractor shall manage all patient information in accordance with HIPAA standards, Privacy Act, and IHS Service Unit and/or Health Center specific policies and protocols.

- 5.6.1. The Contractor shall ensure adherence to DHHS IT system security policies and procedures. The IT policies and procedures will be made available to the Contractor.
- 5.6.2. The Contractor shall immediately report to the Contracting Officer or COR any information or circumstances that may violate any statute, policy, or procedure.

5.7. **IHS Information Technology Systems:**

- 5.7.1. In performance of this contract, the Contractor shall adapt to and successfully utilize IHS information technology systems that are deemed by the Supervisor as necessary for acceptable contractor performance.
- 5.7.2. The Contractor shall ensure that IHS information technology system security policies and procedures are adhered to. The IT policies and procedures will be made available to the Contractor at each IHS facility.
- 5.7.3. The Contractor shall follow all appropriate federal and locally required information Technology security requirements and will be operating under Health Insurance Portability and Accountability (HIPAA) and Privacy Act regulations.

6.0 **Contractor Qualification Requirements:**

- 6.1. **Professional Development.** Degree or diploma from a professional nursing program approved by the legally designated State accrediting agency at the time the program was completed by applicant.
- 6.2. **Experience.** The Contractor shall have thirty-six (36) months of nursing experience, with a minimum of twelve months (12) in the emergency department by the contract start date, unless otherwise approved by the Contracting Officer. The Contractor shall have experience that equips the applicant with the particular knowledge, skills and abilities to perform successfully the duties of the position, and that is typically in or related to the work of the position to be filled.
- 6.3. **License/Registration.** All nurses shall possess a current, valid, unrestricted nursing license in a state, the District of Columbia, the Commonwealth of Puerto Rico, or a Territory of the United States, throughout the term of this contract.
- 6.4. **Certifications (American Heart Association).** Current Basic Life Support (BLS), Pediatric Advanced Life Support (PALS), Advanced Cardiac Life Support, PNCC are mandatory.
- 6.4. **Health Requirements/Conditions of Employment:**
 - 6.4.1. **Medical Evaluation.** The Contractor shall provide a fitness for duty certificate issued by a licensed physician to perform the proposed job without significant risk to personal health or the health and safety of others if required.
 - 6.4.2. **Immunization.**
 - A. Hepatitis A - 2 dose series at 0 and 6 months.

- B. Hepatitis B - 3 dose series at 0, 1, and 6 months or documented immunity. A signed declination of Hepatitis B vaccination series will be accepted.
- C. Measles, Mumps, Rubella (MMR) - 2 doses or titer documentation.
- D. Varicella (chickenpox) - 2 doses, titer documentation or history of varicella/shingles.
- E. Tetanus, diphtheria, pertussis (Tdap/Td) - 1 dose of Tdap, then tetanus (Td) booster every 10 years thereafter.
- F. Meningococcal - Two vaccines (MenACWY and MenB) recommended for microbiologists routinely exposed to *N. meningitides*.
- G. Documentation of annual TB test - may include Mantoux TB skin test (PPD). All new employees will require the 2 step TST. Acceptable lab testing: Quantiferon Gold or T-Spot test. For positive TB test results, please submit a copy of recent chest x-ray results.
- H. Influenza - annual vaccination.

6.5. **Language Requirements and Cultural Awareness.** The Contractor shall read, understand, speak, and write English to effectively communicate with patients and other health care workers, and shall be respectful of the local, American Indian and Alaska Native culture.

6.6. **Information Technology Skills.** The Contractor shall possess knowledge, skills, and abilities to use a computer including EHR software.

6.7. **Orientation:** All nurses providing service under this contract shall attend mandatory orientations and training specified by the government.

6.8. **Background Checks:** As directed by the Contracting Officer, the Contractor shall provide all requested information necessary to perform Level I and Level II background checks. The Contractor shall comply with the requirement to obtain security investigations. The Contractor shall work with the IHS to ensure that the pre-employment screening process includes the appropriate investigation questionnaires and forms to be completed. All completed forms will be reviewed by the Contractor and forwarded to the Supervisor. The Contractor will be immediately removed from the position if at any time the investigation receives unfavorable adjudication, or, if other unfavorable information that would affect the investigation becomes known.

7.0 **Federal Tort Claims Act (28 USC 1346 (B) 1946):** FTCA is not extended to Non-Personal Service Contractors, refer to clause 52.237-7 Indemnification and Medical Liability Insurance.

8.0 52.249-12 Termination (Personal Services) (Apr 1984):

The Government may terminate this contract at any time upon at least 15 days' written notice by the Contracting Officer to the Contractor. The Contractor, with the written consent of the Contracting Officer, may terminate this contract upon at least 15 days' written notice to the Contracting Officer.

8.1. Any Contractor employee demonstrating impaired judgment shall not be allowed to continue working immediately on the contract. The Government reserves the right to remove from the facility any Contractor employee who in the judgment of a licensed healthcare practitioner is impaired.

10.0 <u>Performance - Q A S P based Matrix</u>				
Performance. based Task	Indicator	Standard	Quality Assurance	Incentives
State the end results or outputs that you, the customer will formally accept or reject.	For the requirement, state the feature(s) of end result that will be surveilled.	For each "indicator," state a performance level that, when met, means the task has been performed satisfactorily. This Standard describes "What Success Looks Like."	For each "Standard", state the method used to check performance (i.e. random sampling, 100% inspection, periodic inspection, customer complaints).	List Positive and Negative Incentives. Address method of linking payment to quality of service.
Contractor shall provide Nursing services in the delivery of patient care to the Navajo Area Indian Health Service.	Competency Compliance Patient Outcomes Professionalism Credentialing Documentation Service Quality	Perform 100% required tasks at 100% of the required competencies (refer to 11.0) 100% compliance with IHS Service Units and/or Health Centers published Policies; Procedures; Standards of Care; and hospital and nursing Protocols. No reports of breached patient safety, infection control, and other procedures that might adversely affect patient outcome. Performance characterized by continual cultural awareness and focus on customer service. 100 percent adherence to the Nursing Code of Ethics and Federal Code of Conduct. Uninterrupted credentialing as defined in 6.0 to 6.8 for period of contract. 100% percent of all documentation meets established IHS Medical Facility, JCAHO, and CMS standards to include, but are not limited to, timeliness, legibility, accuracy, content, date, time, signature and designated profession. Satisfaction with quality of service is evidenced by valid customer inputs.	See GASP. Surveillance systems will include periodic inspections and customer complaints.	Payment of contract price for satisfactory service. Contractor performance will be evaluated using the Contractors Performance Assessment Report (CPAR). The evaluation will be considered when future IHS Contract selections are made.